



Bromocriptine

 **Tablet**

DESCRIPTION

White, round, biconvex tablet with 'RMB' breakline on one side and plain on the other.

COMPOSITION

Each tablet contains Bromocriptine mesylate equivalent to Bromocriptine 2.5 mg.

ACTIONS

Bromocriptine, an ergot derivative or ergoline is a dopamine D₂-antagonist which has antagonist properties at D₁ receptors. It inhibits the secretion of prolactin from the anterior pituitary.

Growth-hormone secretion may also be suppressed by bromocriptine in some patients with acromegaly. Bromocriptine has dopaminergic activity.

Bromocriptine is incompletely absorbed from the gastrointestinal tract, metabolised in the liver and mainly excreted in the bile. It has been reported to be 90 to 96% bound to serum albumin *in-vitro*.

INDICATIONS

Bromocriptine is used in endocrinological disorders associated with hyperprolactinaemia, including amenorrhoea in women and infertility in men.

Growth-hormone secretion may also be suppressed by bromocriptine in some patients with acromegaly. Because of its dopaminergic activity bromocriptine is also used in the management of Parkinson's disease.

CONTRAINDICATIONS

Hypersensitivity to bromocriptine or any other ergot alkaloids, toxemia of pregnancy, hypertension post-partum or in the puerperium.

ADVERSE REACTIONS

Nausea is the most common side effect at the beginning of treatment with bromocriptine, but vomiting, dizziness, and orthostatic hypotension may also occur. Syncope has followed initial doses of bromocriptine.

Side effects are generally dose-related and may therefore be more frequent with the higher doses that have been used in the treatment of parkinsonism and acromegaly. Reduction of the dosage of bromocriptine, followed in a few days by a more gradual increase, causes the reversal of many side effects. Nausea may be diminished by taking bromocriptine with food.

Bromocriptine is a vasoconstrictor; digital vasospasm, induced by cold, and leg cramps have been reported especially with high doses. Other cardiovascular effects have included erythromelalgia, prolonged severe hypotension, arrhythmias, and exacerbation of angina. Bromocriptine is associated with somnolence and has been associated very rarely with excessive daytime somnolence and sudden sleep onset episodes.

Other side effects reported include headache, nasal congestion, drowsiness, dryness of the mouth, constipation, diarrhoea, and altered liver-function test. Dyskinesias have occurred in patients suffering from parkinsonism. Gastrointestinal bleeding has been reported in acromegalic patients. Psychosis, with hallucinations, delusions, and confusion, occurs particularly when high doses are used to treat parkinsonism; but has also been reported with low doses.

PRECAUTIONS

Patients with hyperprolactinaemia should be investigated for the possibility of a pituitary tumour before treatment with bromocriptine. Treatment of women with hyperprolactinaemic amenorrhoea results in ovulation; such patients should be advised to use contraceptive measures other than an oral contraceptive.

Acromegalic patients should be checked for symptoms of peptic ulcer before therapy and should immediately report symptoms of gastrointestinal discomfort during therapy.

Bromocriptine should be given with caution to patients with cardiovascular disease, liver disease, or a history of psychotic disorders.

Annual gynaecological examinations or every 6 months for postmenopausal women are recommended and blood pressure should be monitored periodically.

Pregnancy

Bromocriptine is generally not recommended for use during pregnancy. However, large and longterm studies performed in humans have found no increased incidence of birth defects. With clinical usage successful pregnancies have occurred in humans with administration of bromocriptine both before conception and for periods ranging from the first 2 to 3 weeks to the full longterm of pregnancy.

Breastfeeding

This medication should not be administered to mothers who intend to breast feed, since bromocriptine interferes with lactation.

Paediatrics

Studies performed to date have not demonstrated paediatrics - specific problems that would limit the use in children.

Special warning & Special precautions for use:

Bromocriptine has been associated with somnolence and episodes of sudden sleep onset, particularly in patients with Parkinson's disease. Sudden onset of sleep during daily activities, in some cases without awareness or warning signs has been reported rarely. Patients must be informed of this and advise to exercise caution while driving or operating machines during treatment with bromocriptine. Patients who have experienced somnolence and/or an episode of sudden sleep onset must refrain from driving or operating machines. Furthermore a reduction of dosage or termination of therapy may be considered.

Effects on Ability to Drive and Use Machines

Patients being treated with bromocriptine and presenting with somnolence and/or sudden sleep episodes must be informed to refrain from driving or engaging in activities where impaired alertness may put themselves or others at risk at serious injury or death (e.g operating machines) until such recurrent episodes and somnolence have resolved (see also section on special warnings and special precautions for use).

Drug Interactions

Drug	Effect
Alcohol	Disulfiram - like reaction may occur.
Oral contraceptives, oestrogens or progestins	Interference with effects of bromocriptine; concurrent treatment not recommended.
Haloperidol, loxapine, methyl dopa, metoclopramide, molindone, MAO inhibitors, phenothiazines, reserpine or thioxanthenes	May increase serum prolactin concentrations, dosage adjustment of bromocriptine may be necessary.
Levodopa	Additive effects allowing reduction in levodopa dosage.
Erythromycin	Concomitant administration has been reported to significantly increase the bioavailability of bromocriptine.

DOSAGE AND ADMINISTRATION

Doses are expressed in terms of the base.

Tablets are to be taken with food.

Hypogonadism, galactorrhoea syndromes, infertility in men and women, cyclical benign breast and menstrual disorders:
Initial dosage of 1.25 to 2.5 mg at bedtime with a snack. Dosage may be increased by 2.5 mg every 3 to 7 days, as needed to a total of 5 to 7.5 mg a day taken in divided doses with meals.

Hyperprolactinaemia, prolactinomas:

Initially 1.25 mg two or three times a day with meals. Dosage adjustment is gradual over several weeks 10 to 20 mg a day taken in divided doses with meals. Occasionally higher doses may be required.

Acromegalic patients:

Initially 1.25 to 2.5 mg at bedtime for 3 days; dosage may be increased by 1.25 or 2.5 mg every three to seven days up to 30 mg a day taken in divided doses.

Parkinson's disease:

Usually bromocriptine is an adjunct to levodopa treatment

Bromocriptine is gradually introduced, during which time dosage of levodopa can also be decreased gradually, until an optimal response is achieved. A suggested initial dose is 1.25 mg one or two times a day; for single doses, at bedtime with a snack is preferred. Dosage may be increased by 2.5 mg increments every fourteen to twenty eight days. Maintenance doses range from 10 to 30 mg a day taken in divided doses to a maximum of 100 mg a day.

OVERDOSAGE

Symptoms:

Lethargy with altered mental status, hypotension, shallow breathing, dilated pupils and hyper-reflexic lower extremities.

Treatment:

Induction of emesis and/or gastric lavage followed by activated charcoal and magnesium citrate suggested.

ROUTE OF ADMINISTRATION

Oral.

STORAGE CONDITIONS

Store below 30°C.

Keep container tightly closed.

Protect from light.

SHELF LIFE

Product should not be used beyond the expiry date imprinted on the product packaging.

PRESENTATION

In blisters of 30, 60, 100, 120, 250 and 500 tablets.

In bottle of 100 tablets.

Date of revision: 04th August 2017

PRODUCT REGISTRATION HOLDER/ MANUFACTURER:

PHARMANIAGA MANUFACTURING BERHAD (60016-D)

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